
CONSULT FOR AFRICA

The Onboarding Pack

Office of the Founding Partner

Prepared for Ezinne Orji, Executive Assistant to the Founding Partner

Consult for Africa internal. Not for circulation outside the firm.

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hello@consultforafrica.com / +234 913 813 8553 / consultforafrica.com

Lagos and Abuja, Nigeria



A letter from Debo

Ezinne,

Welcome, and thank you for signing. I am glad this is settled, because the office has been running without the person it was designed around and I have felt it every week since.

I want to tell you what you have actually joined, because the shape of this firm is not obvious from the outside and you would otherwise spend your first month inferring it. Consult for Africa is a healthcare management consultancy, but we are not the kind that writes a report and leaves. We are hired by hospital owners, group chief executives, family offices and boards, usually because they have built something that is not performing and cannot see why, or because they are about to commit serious money and want somebody who has run a hospital to tell them the truth before they sign, or because they need somebody to actually run the thing. Most of our engagements start as the first and turn into the second or the third, which is why almost every piece of work described in this pack is larger now than it was when it began.

Around that core we have built products, and the reason matters. The same three things break in every African health institution we are called into, which are leadership, workforce and the economics of a practice, and we got tired of recommending fixes that nobody in this market could actually buy. So we built them.

A few things I want to say plainly up front about the job.

Your role exists because I am the bottleneck. Work reaches me, waits for me, and sometimes dies there, and the cost of that is not visible in any report because nobody writes down the proposal that went out nine days late. The point of this office is to make my attention the scarce thing it actually is and to stop it being the thing everything queues behind. You are not here to tidy my diary. You are here to hold the follow-through for a firm that is carrying more engagements than it has people, and to make sure that a promise made in a meeting in Lagos on Tuesday is still being chased in Abuja on Thursday.

Abigail Oladejo reports to you, not to me. She joined us from university, she is quick and she wants to learn, and she has held this office on her own since she started. Everything she is given carries a written brief, a definition of done and an example of one that is already right, and I expect you to hold that standard even when explaining it verbally would be faster. It is what makes the difference between work that lands and work that has to be done twice, and that is a statement about how we brief rather than about her. When she is stuck she marks the work blocked, which comes to you rather than to me. If I find myself reviewing her output directly, something has gone wrong in the chain and I will say so.

You are based in Abuja and that is deliberate rather than convenient. Our registered working base is in Lifecamp, the largest single piece of work starting this month is in Asokoro, the Medbury campus conversion we have been mandated to set up spans Lagos and Abuja, and the institutional conversations that shape this sector happen in Abuja whether we are in the room or not. Part Four of this pack is the longest part for that reason.

The last thing is confidentiality, and I am strict about it. Everything in this pack is real, including the money, the client names and the findings, because you cannot hold the follow-up on work you do not understand. None of it leaves the firm. We describe our work by method and lesson, never by client, and in a market this small a combination of harmless details identifies somebody.

You will not absorb all of this at once and I do not expect you to. You start on Friday 2 October, which is also day one of the Arabella engagement in Asokoro, so read Part Four first and before Friday if you can, then Parts One to Three over the weekend and the start of next week, and treat Parts Five to Seven as the reference you return to.



The natural first step is for you and me to sit down once you have read Part Four, and go through Arabella and the Medbury division in detail, because those two carry the most risk between now and Christmas.

Warm regards,

Dr Debo Odulana

Founding Partner, Consult for Africa

How to use this pack

Part	What it covers	When you need it
1	The firm: what we sell, how we are bought, who is here	Day one. Read once, properly.
2	Your role, the delegation chain, and how this office runs	Day one, alongside Part One.
3	The platform: your surfaces, and the state they are in today	Day two, in front of the screen.
4	Abuja: every live and forming piece of work in the city	First. Arabella is live from today.
5	The rest of the book of work, Lagos and the products	Week one, then as reference.
6	Money: what is owed, what you chase, what you never touch	Week one.
7	How we write, and what never leaves the building	Before you write anything.
8	Your first ninety days	Week one, then again at week four.
A to D	Directory, glossary, standing checklists, where things live	Whenever you are unsure.

One convention, and one deliberate absence. Where a fact needs confirming rather than knowing, it says so in the text, and those are flags rather than gaps I forgot to fill.

The absence is money. **This document carries no fees, no balances and no client commercials**, by Debo's instruction. Fee structures are the most sensitive commercial information the firm holds, and a document travels in a way a login does not. Every figure you need is on the platform under Invoices, which your role reaches, and the engagement record carries the fee structure and the payment schedule for each client. Where this pack tells you to chase something, read the number off the platform on the day you chase it, which is also the only way to be sure it is current.

Part One: The firm

1.1 What Consult for Africa is

Consult for Africa is a healthcare management consulting firm founded in 2024, working from Lagos and Abuja across Nigeria and into the wider continent. It is written **Consult for Africa** in body copy and **CFA** in short form, and you will also see **C4A** in older files and in file names. Both are us, and CFA is preferred in anything new.

The legal billing entity is **Consult for Africa Management Services Limited**, and that is the name on invoices and contracts. It does not appear in marketing copy.

The one line version, which is the one to use:



Consult for Africa helps hospitals, health systems, investors and governments turn ambition into delivery. Built by clinicians and operators who know the system from the inside, we do not hand you a report and walk away. We build, staff, and stand behind the outcome.

The tagline on the brand guide is **Healthcare performance, restored.**

1.2 The argument the firm rests on

Across the continent the gap between a struggling hospital and a thriving one is rarely the building or the equipment. It is execution, leadership and people. That is not a slogan somebody reverse engineered for a website, it is what every engagement has found independently, which is why a paediatric centre that looked like it had a stock control problem turned out to have an incentives problem, and a surgeon who believed his growth was limited by his clinic's postcode turned out to be limited by a referral pipeline he had never built.

You will hear Debo put it more bluntly, and this version is worth remembering because it decides how our reports are written: the presenting cause is always a thing, and the reason is never the thing.

1.3 What we are actually bought for

The published service lines are the menu. These five are what clients actually sign, and knowing the difference tells you what a piece of work will demand from the office.

What it is	What it looks like week to week	Example here
The organisational audit	A structured diagnostic across ten domains, a site visit, surveys of staff, patients and referrers, then a report with findings and decisions for the board	Osteon, Arabella, Haven phase 2
Turnaround and growth mandate	A target, a set of levers and delivery against it, usually with us inside the business	Haven phase 3, House of Refuge
Board governance	Standing up a board, its committees, delegation of authority, director training	Havana Specialist Hospital
Venture structuring	Holding companies, joint ventures, cap tables, shareholder agreements, alongside the client's counsel	Kemi Balogun group, the aesthetics SPV
Build and operate	We design the facility or the venture, set it up, then manage it under a management agreement	Medbury division, Lyfe Place Abuja, ile

The seven published service lines sit on consultforafrica.com and are worth reading once: Clinical Governance, Strategy and Growth, Fractional Leadership, Digital Health, Health Systems Strengthening, Diaspora Expertise, and the Healthcare Marketing Agency.

1.4 The progression, and why your diary depends on it

There is a pattern that recurs often enough to plan around. A client buys a small diagnostic, the diagnostic finds something larger than the brief, and the next conversation is about a mandate. Arabella is the clearest live example, because the audit starting this week is explicitly the qualification round for a management contract proposed from month three that is worth a multiple of the audit over a year. The audit is what is sold; the contract is what it is for.

What that means for you is that a diagnostic deadline is never only a deadline. It is an audition, and the client is forming a view of whether we are the people to hand the business to. When you are deciding what to chase first, the work that is being watched beats the work that is merely due.



1.5 The four brands and the ventures

Consult for Africa is the firm and the consulting brand. Everything else sits under it.

Maarova is leadership assessment, psychometrics and executive coaching built for African healthcare, with a Founding Circle of hand picked clinical leaders. It is sold into hospitals as leadership assessment and development rather than as software.

CadreHealth is the healthcare workforce platform at oncadre.com, holding a bank of over ten thousand professional records, and it is how we source clinical and non clinical staff for clients and how we run recruitment mandates.

Mezo Health is a verified specialist network and hospital operating stack, sold to doctors as a private practice opportunity, and client side it is described as our facility operations practice.

ile is a residential eldercare venture being built with a client, written in lower case with the accent, and it is deliberately kept away from CFA branding because a family arriving at ile has no reason to have heard of us.

One rule that matters more than it looks. **Never name the platforms in client body copy.** Describe what is done instead, so a client document says that staff will be trained on a secure client record system, not that we are deploying a named product. The exception is an appendix a client has specifically asked for.

1.6 Who is here

Eighty two people hold platform accounts including you, of which sixty nine are consultants on the bench rather than staff. The names you will deal with weekly are short.

Person	What they hold
Dr Debo Odulana	Founding Partner. Your principal. Former CEO of Cedarcrest Hospitals Abuja, former CISO at Evercare Lekki, founder of Doctoora, and President of the Doctors Foundation for Care
Abigail Oladejo	Administrative Assistant, Office of the Founding Partner. Reports to you
Toluwanimi Olayeni, called Nimi	Marketing and communications lead, and she runs the healthcare marketing agency as a service line
Ejiro Alechenu	Marketing, joined September 2026, works to Nimi
Oludayo Olugbamigbe	HR Practice Lead, joining from the Nigerian Exchange Group
Dr Olufunbi Okeleye	Engagement Manager
Kola Momodu	On site Project Owner for Arabella, five days a week in Asokoro. Independent contractor, ISO 27001 lead auditor, eight years at Cedarcrest Abuja

There are eight directors on the platform, including Matthew Aloba, Dr Adediwura Okeleye, Barr. Osahon Omoruyi, Bukunola Oyedele, Irene Nwaukwa, Olufemi Shonowo, Patience Ugbaja and Tito Ipinmoye. Several hold practice or faculty roles rather than full time posts, and part of your first fortnight is confirming titles and reporting lines with Debo so the directory in Appendix A is correct rather than inherited.

Part Two: Your role, and how this office runs

2.1 What the role holds



The role is the diary, the inbox, travel, meeting preparation and minutes, and the follow up chasing across everything the firm has in flight. Alongside that you carry client work, because everybody at CFA works, and that is why your platform access reaches the delivery surfaces and not only the office ones.

Four things are yours specifically rather than shared with the Administrative Assistant, and they are the ones that make the role senior. You can press send on a communication or a campaign, you can read finance so you can chase money, you hold organiser rights on meetings so you can reschedule, cancel and control the recording bot, and you assign and review work.

2.2 The delegation chain

Work flows from Debo to you to Abigail, and review flows back the same way. He reviews you, you review her, and if he finds himself reviewing her work directly that is treated as a fault in the chain rather than a shortcut, because it undercuts you and puts him back in the loop the office exists to get him out of.

This is built into the platform rather than left to good intentions. The person who assigns a task is the person who reviews it, so when Abigail marks something blocked it raises to you. You can break one of Debo's tasks into sub tasks for her, and he sees the roll up on his original without being in the detail.

2.3 The rules of the task system

These are not suggestions, they are the standard the office is judged on.

A task goes out with a **brief** explaining why it matters and what it feeds into, and a **definition of done** describing what finished looks like in the assignee's words rather than yours. Neither is written verbally and neither is skipped because the task seems obvious.

Anything new to the assignee gets a **worked example** attached the first time. A definition of done describes a finished thing, and if the person has never seen one, you have described something they can only guess at. The example is what turns the standard from a sentence into something they can match.

A **check in date** is set at the moment of assignment and is not renegotiated later, so a five day task gets a day two conversation instead of a day five surprise.

The assigner puts an **estimate** on the task and the assignee can see it. If the real time comes in at triple, the estimate was wrong and both people learn something.

Two statuses carry most of the weight. **Blocked** lets somebody stop without interrupting anyone and without it reading as failure. **Changes requested** sends work back with a written note rather than having it silently redone, which is the habit that actually kills delegation, because an unwritten correction has to be made again next month.

The measure of whether this office is working is not how much gets done in month one. It is whether the volume of work going out with written briefs rises while the volume coming back for changes falls.

2.4 What you can reach, and what is deliberately withheld

Your access was granted surface by surface rather than inherited from a role list, which is why some things you might expect are absent on purpose.

Yours: tasks, campaigns, commitments, decisions, the pipeline, communications, meetings, the operating rhythm, invoices and payment status read only, knowledge, the asset library and the office inventory.

Withheld for now, to be revisited at the three month mark: finance write access, consultant rates, agent commissions, the admin area including users and assessments, Maarova coaching records and Founding Circle applications because those are personal development records, client contracts and non disclosure agreements, and the partner, client, agent and consultant portals.



Nothing in that second list is a comment on you. It is how the office was designed, so that access is always granted deliberately and the places where your role and Abigail's differ stay visible.

2.5 The operating rhythm

Five standing items already exist on the platform, and they generate real tasks on a cadence so that a missed month shows up as a gap rather than as silence. Four currently sit with Abigail and you review them. The fifth has been waiting for you.

When	What	Who	What finished looks like
Monday, packed Friday	Partner meeting pack	Abigail	One document holding every active project and its status in a line, anything that slipped, decisions needed from the partners, and the diary for the week ahead
Thursday	Follow up sweep across the pipeline	Abigail	Every lead untouched for fourteen days chased or annotated, every proposal older than a week followed up, anything needing a partner on the commitment register with a name against it
The 1st	Invoice run	Abigail	Every engagement with billable work has an invoice raised and approved, anything that cannot be invoiced has a written reason, and the list is with Debo
The 15th	Overdue invoice chase	Abigail	Every invoice past its due date chased, with the date the client promised to pay recorded on the commitment register, and disputes escalated rather than chased again
The 28th	Monthly evaluation, Office of the Founding Partner	You	An evaluation recorded against the open rotation with all five scores, at least one strength and one development area written in specifics rather than adjectives, and the conversation held before the form is filled

The monthly evaluation was written to sit with Debo only until this hire landed, so it has moved to you with effect from this month. The four recurring templates now carry you as the assigner, which means from the next cycle Abigail's work comes to you for review rather than to him.

2.6 How Debo works

He is fast, he wants things done rather than discussed, and he does not want trailing summaries. A short, specific update beats a complete one. If a thing is decided, say what was decided; if it is stuck, say what you need from him in one line.

He reads on his phone between meetings, often late. Anything that needs a decision should be able to be answered with a yes, a no or a number, and if it genuinely cannot, it belongs on the decisions register with the options and your recommendation written out rather than in a message.

He often knows the person you are writing to, frequently from Cedarcrest, Evercare or Doctoora. **Always ask how he knows somebody before you draft anything that goes to them**, because a correct but cold letter to a friend of fifteen years costs more than a late one.

2.7 The three month mark

At three months the withheld list gets revisited, which in practice means finance write access and some of the admin surfaces. What earns that is not tenure. It is the review backlog staying empty, the chase register being current, and work coming back finished because the briefs going out are good.

Part Three: The platform



3.1 Getting in

The platform is at www.consultforafrica.com/login and your account is ogeriorji845@gmail.com, created on 1 October 2026 with the role Executive Assistant. A temporary password was emailed to you from hello@consultforafrica.com, and you should change it at the first login from Settings.

If a page ever tells you that you do not have access, check which account you are signed into before assuming the page is broken. This has cost time before, because Debo himself holds two accounts and only one of them reaches the admin pages.

3.2 My desk

Your landing page is **/desk**, and it answers one question: whose move is it. It sorts everything into four headings, which are work that is yours now, work waiting on other people, work waiting on a date rather than a person, and reference that is waiting on nobody.

Work down the list. If the desk is empty and it is a Thursday, something has not been generated and that is itself worth checking.

3.3 Your surfaces, and what good looks like on each

Tasks at **/tasks** is the board, filtered to you. You assign from here, and you review from here.

Commitments at **/commitments** is a promise somebody made and the record of chasing it. It is deliberately not a task, because a task is work the firm holds and can assign, while a commitment is usually owed by somebody we cannot assign to, which is exactly why it gets forgotten. It holds who owes it, who it is owed to, when it was last chased and how many times, so that a commitment nobody has chased and a commitment chased four times read as the different problems they are.

Decisions at **/decisions** is where a choice that belongs to Debo gets put in front of him with its background, its options, your recommendation and the cost of leaving it alone. That last field exists so the price of a deferral is visible at the moment of deferring it.

Pipeline at **/pipeline** holds leads, discovery calls and proposals. You own the hygiene and the chasing on it, not the deal terms.

Communications at **/communications** is the client relationship record: calls and emails logged, next actions set, templates kept. You can send from here and Abigail cannot, which is deliberate.

Campaigns at **/campaigns** is outreach list building and sending.

Invoices at **/finance/invoices** is read only and exists so you can chase. You will not see rates or commissions and you cannot change a figure.

Meetings at **/meetings** carries agendas, minutes, participants and the recording bot, and you hold organiser rights.

Rhythm at **/rhythm** is where the five standing items above are configured.

Knowledge and the **Asset Library** are the firm's reference material and reusable deliverables.

Inventory at **/inventory** is the office asset and stock register.

Nuru at **/ai** is the firm's assistant, useful for drafting and for interrogating what is on the platform.

3.4 The state of these surfaces today, which is the honest version

Some of this is yours to fix, and the list is short enough to clear in your first fortnight.

The **commitments register** is **empty**, and so is the **decisions register**. Both were built for this office and neither has ever been used, which means the promises made in meetings since July exist only in people's



memory and in meeting notes. Rebuilding that from the meeting records and the WhatsApp threads is the single highest value thing you can do in week one, because four of the five standing rhythm items depend on a register that currently has nothing in it.

There are **no meetings in the diary on the platform**, which does not mean there are no meetings. It means the diary is being kept elsewhere.

The **pipeline holds one lead**, dated 18 September, from a Maarova demo request by Dr Moromoke Ogunko at Ifako Ijaiye General Hospital in Lagos. Nobody has touched it. Four proposals sit in draft, two of them for SafeCare and one for Duchess International Hospital, all raised in March and never sent from the platform even where the real document went out by email.

The **office inventory is at zero**. The first count has been in progress since 23 September.

Two tasks have been sitting submitted and unreviewed since mid September, and the Monday partner meeting pack has generated three times without being completed. That backlog is the clearest illustration of why this role exists, and clearing it with Debo rather than silently is a better first move than closing it quietly.

Several client records are **missing the contact details** that any send or portal invitation would need. Dr Chito Nwana at Arabella has no email or phone on record anywhere, Dr Bola Akinola at Osteon has neither, and Havana needs contact records for Ugo Nwokoro and for Mrs Nneka Chukwubuisi Solomon in finance. These were left blank rather than guessed, and filling them is a genuine piece of work rather than data entry, because an invitation sent to a guessed address is worse than one not sent.

3.5 Two commercial facts that are not on the platform yet

The Medbury division deal closed on 21 September, with a setup fee in tranches and a monthly support fee from month six, and **there is no engagement record for it**. The only Medbury record on the platform is the Lyfe Place campus engagement, which is paid.

House of Refuge runs a monthly management retainer and a fundraising mandate on a success fee, and **neither strand exists as an engagement** either. Only the completed readiness project is on the record.

Flag both to Debo in week one. Until they exist, the invoice run on the 1st of the month cannot see them, which is how a retainer quietly goes unbilled.

Part Four: Abuja

4.1 Where you sit, and why the city matters

Our working base on the client record is **Lifecamp, Abuja**, and that is where you are. Lagos carries more of the current fee income, but Abuja carries the work that is starting, the work that is largest, and the institutions that decide what this sector is allowed to do.

Three things make the city strategic rather than incidental. The biggest single engagement beginning this month is in Asokoro and it is the gate to an annuity rather than a one off fee. The Medbury mandate we closed in September covers Lagos **and** Abuja, which is the argument that got the monthly fee to where it is, and it doubles every registration we have to obtain. And the regulators, the Corporate Affairs Commission, the Federal Inland Revenue Service, the Medical and Dental Council, the data protection regulator and the Ministry of Health all sit here, so a filing that takes three weeks by post takes an afternoon by someone walking in.

Debo moves between the two cities. Assume he is in Lagos unless the diary says otherwise, and build Abuja meetings into blocks rather than scattering them, because a single Abuja day that holds four meetings is worth more than four trips.



4.2 Arabella Women's Health, Asokoro

This is the most important thing on your desk and it began today.

What it is.

Arabella Women's Health is a women's health facility in Asokoro, Abuja, led by **Dr Chito Nwana**, who is both the principal and the lead clinician. Her Chief of Staff is **Tolu**, who keeps her existing role. The engagement had been in negotiation since April 2026 and closed on Monday 28 September 2026.

Arabella is a **new entity with its own CAC registration and RC number**, and it succeeds **Tabitha Medical Centre**. That succession is the single most important fact in the file, because it decides what moved across and what did not, including the patient records, the staff contracts and the health insurance panels.

The scope, and this is the part to get right.

We are doing the diagnostic audit, and nothing else. That is Debo's instruction and it is the sentence to hold on to, because the proposals Dr Chito already holds describe a great deal more than an audit.

Those earlier documents set out a two month foundation phase that included a digital marketing strategy and setup, a public relations strategy with press materials, a pharmacy inventory audit with supply onboarding, the deployment of a client records and operations system with all patient facing staff trained inside a fortnight, a successor health insurance introduction programme, a seconded Head of Quality and Clinical Operations, a reactivation campaign against the Tabitha patient database, and a live dashboard refreshed every twenty four hours. **None of that is what we are delivering now.**

So the single largest risk on this file is not delivery. It is **the gap between what she has read and what we are doing**, because she will reasonably expect the former and nobody has written down that the scope is narrower. Your job is to make sure that gap is closed in writing rather than discovered in week five when she asks where her marketing strategy is. Raise it with Debo in your first conversation, get the in scope list confirmed, and get it to her as a short note rather than letting the proposal stand as the description of the work.

When she or Tolu refer to something from that list, do not improvise and do not agree to it. Note it, and bring it back.

What the audit actually is.

The audit is a structured diagnostic and it has four moving parts, all of them already built.

- The **information request**, a ten page document with thirteen lettered sections, A to M, and eight priority items at the front. Section A is the succession question and is the one that matters most: the two entities, what moved and what did not, and custody of the patient records.
- An **uploader** against each section, so a document she sends is tagged to the question it answers rather than landing in a mailbox.
- **Four survey instruments**, covering staff, patients, referrers and leadership.
- The **written operations and commercial diagnostic**, covering revenue by payer, billing leakage, pharmacy stock and formulary gaps, staff task compliance and database quality, and then the report.

The sections that are specific to this client rather than generic are worth knowing, because they are where the findings will come from: the embryology laboratory with its cryostorage consents and tank alarms, the obstetric escalation route in writing with the named intensive care and neonatal unit, what a maternity package excludes and what happens commercially on a caesarean or a transfer out, insurance receivables ageing with rejection reasons, and the lawful basis for contacting the Tabitha database.

That last one is the question with the longest reach. The whole growth case rests on reactivating a patient database of over three thousand records, and if there is no lawful basis for contacting those women then the



reactivation is not a marketing exercise, it is a data protection problem. It sits in section A for that reason. Gather the answer rather than letting it drift, even though the campaign itself is not ours to run yet.

The money, and the two things to confirm.

The audit is invoiced as a mobilisation payment, received on 28 September, and a balance dated to month two. Invoice CFA-ARA-2026-001 sits on the platform as partially paid and the engagement runs 2 October to 30 November 2026. Read the amounts there rather than from any document.

Two things need confirming and both are quick. **What the fee covers**, in one written list, for the reason set out above. And whether the invoice Debo sent carries **the non refundable term, the free substitution right and the one free reschedule**, because the platform record carries those terms in its notes and the emailed document was raised outside our systems, so it may not. That matters here specifically: a client has asked us for a refund before on an invoice that was silent on the point.

Why the audit is worth more than its fee.

The forward path, and it is a proposal rather than an agreement, is a **management contract charged as a share of monthly revenue from month three with a floor in the early months**. One number a month, no tiers, no itemised invoice. Against even the conservative revenue case that is worth a multiple of the audit over a year.

So the audit is the qualification round. Every deliverable over the next eight weeks is being read by somebody deciding whether to hand us her business, which is why the quality of the report matters more than the size of the invoice, and why a missed deadline on a small piece of work is expensive out of all proportion to it.

Treat the rest of the commercial as background you understand and do not quote. The insurance successor programme, the reactivation sprint, the secondment and the dashboard all belong to a phase that has not been sold.

What we need from Dr Chito.

Two documents, and both are audit evidence rather than paperwork.

1. The Arabella **certificate of incorporation and RC number**, which is where the succession question starts
2. A list of the **health insurance panels Tabitha currently holds**, with tariff rates where available, which is how the payer and receivables picture gets built

A third item, her signed introduction letter as lead clinician, appears in the proposals and belongs to the insurance successor programme. **It is not needed for the audit**, so do not chase it, and if she offers it, park it.

Ask in the room rather than by email, because a request made in person on site is answered and the same request by email joins a queue.

Who is on it.

Kola Momodu is our named on site Project Owner. He spent nearly nine years at Cedarcrest Abuja, he is certified as a lead auditor in information security, and he is an independent contractor, which means 5 per cent withholding tax is deducted at source and remitted by us.

The proposals commit him to five days a week physically inside Arabella, which was written for the full foundation phase rather than for an audit. **Confirm with Debo what he is actually doing and for how many days**, because the client has read the larger number and will notice if he is there less, and because he should not be committed to five days of a two month phase that is not running.



The seconded Head of Quality and Clinical Operations in the proposals is part of the same unsold phase, so no candidate is needed now. Worth knowing for when it comes back, since the document described that person as confirmed and briefed before engagement start and named nobody.

What she has been promised she will see.

A live dashboard showing consultations, maternity packages, revenue attributed, pipeline value and reactivation conversion, refreshed every twenty four hours, plus a short brief by message every Monday and a monthly review call with Debo.

The dashboard belongs to the management phase and the route does not exist. Do not promise it and do not let anybody else promise it. What is worth doing inside the audit is the **weekly brief**, because it is cheap, it is the habit the management relationship would run on, and it is the best possible demonstration of what we would be like to work with. Set it up as a standing item.

The one link, and the fact that it has not gone out.

Everything the client needs sits at **consultforafrica.com/ArabellaProject**, which is unindexed and was built so that we send one link instead of six attachments. It carries the information request, the eight priority items, the uploader, the four surveys and a week by week view of the first fortnight.

It is live in production and was verified there on 30 September. **It has not been sent to her, because her email address is not recorded anywhere in our systems.** Debo has her details. Ask him, send the link to her and to Tolu, and log the send.

The four surveys are a staff culture instrument of fifty five scale items across twelve sections, a patient experience instrument of twenty four, a referrer instrument of thirteen plus a hundred point allocation, and a leadership direction instrument of twenty four plus an allocation. The referrer one matters more than its length suggests, because Arabella has no referrer list, which is itself a finding, so the form builds the list as it goes by asking every respondent to name up to three colleagues and asking directly about referring that has fallen or stopped. Responses land on the platform and are read at /admin/surveys. Check the counts weekly, because the Osteon audit sat at zero responses for two weeks and nobody noticed.

Your Arabella chase list

- Dr Chito's email address and phone number from Debo, onto the client record
- The ArabellaProject link sent to her and to Tolu, and logged
- The in scope list confirmed with Debo and put to her in writing
- CAC certificate and RC number
- Current insurance panel list with tariffs
- What Kola is actually committed to, confirmed and consistent with what she has been told
- Non refundable terms confirmed present on the issued invoice
- The weekly brief set up as a standing item
- Survey distribution to staff, patients and referrers, with response counts checked weekly
- Uploads arriving against the priority sections, and the silent ones chased
- The month two balance diarised, with the trigger for it confirmed in writing

4.3 Lyfe Place Abuja, and the Medbury division

The client.

Medbury Healthcare Group, group chief executive **Dr Itunu Akinware**. This is the firm's largest commercial relationship and it has three strands, one of which is the Abuja campus.

The campus.



Lyfe Place is a four structure residential compound in Abuja being converted into a private ambulatory medical campus, with a theatre on the ground floor and a family practice above it. Our work recovered the building's **measured geometry** from the as built drawings rather than accepting the brief, and the brief turned out to be wrong. The campus is 953.9 square metres gross and 664.3 net, not the 711 and 519 the brief carried. The extra 145 square metres is all corridor, which means the rent roll is flat while the setup costs materially more. There are eight consulting rooms, not the one the original model allowed for, and the compound has only one external door, so the specimen and clinical waste flows the brief assumes cannot be kept without three new external openings.

Two questions remain open and they change the answer rather than the confidence in it. The wellness catalogue assumes seven hundred and twenty visa medicals a year, which gates a large revenue line, and in Nigeria that volume runs through embassy or international organisation panel physicians, so if Lyfe Place is not on those panels the revenue is unreachable rather than under marketed. That is one phone call and it has not been made. Separately, two of our own documents disagree on sessional fill, one carrying 35 to 55 per cent by band and the other a 75 per cent booking ceiling, which is a 1.8 times difference on the largest revenue line on the campus. Neither can be quoted again until they are reconciled in one place.

On the model, the monthly version is the authority on timing rather than the annual one, and it puts both the payback point and the peak funding requirement further out than the earlier work did. If anybody quotes a payback figure at you, check it against the monthly model before it goes in a document.

The division deal, which closed on 21 September.

This is the commercial spine of the relationship and it covers **Lagos and Abuja**, which is how the monthly fee was argued up.

Term	What was agreed
Setup	A fixed fee over five months across both cities, in four tranches, exclusive of value added tax and exclusive of government and regulatory fees
Government fees	Medbury pays them directly or reimburses at cost with no mark up, and we owe a schedule of expected fees for both cities before the commencement date
The two chief operating officers	Medbury employs and pays them. CFA recruits them at its own cost , and they are seconded to CFA reporting to Debo
Support fee	Monthly from month six , for the shared functions and for the direction of the division toward the board's goals
Performance	A share of collected net revenue , payable only where the board's revenue goal and its margin goal are both met
Term	Twenty four months, reviewed at twelve

Two things about this deal are worth holding in your head, because they affect what you escalate.

The **reporting line is the load bearing clause**. Debo flagged during the negotiation that putting the chief operating officers on Medbury's payroll makes us dispensable, and the answer is that they are seconded to us and report to him, so we have direction without employment. The memorandum says in terms that the arrangement works only while that reporting line is observed in practice as well as in writing. If those two people start taking direction from Medbury, we are an invoice. Watch for it in small signs, such as meetings they are summoned to without us, and raise it early rather than once it is a pattern.



The payment terms were hardened deliberately because late payment puts delivery at risk, and they are unusual enough that you need to know them. The support fee is payable monthly **in advance**, on or before the fifth business day of the month it relates to, and we invoice by the **25th of the preceding month**. Both sides put it on a standing payment instruction so it does not depend on somebody approving it each month. One month's fee is held as a deposit and applied to the final month. A disputed line does not hold up the undisputed part, which must still be paid on the due date, and the dispute must be notified within five business days with reasons. Interest runs at the central bank rate plus three per cent. Late twice in twelve months is a material breach. We may suspend after fourteen days past due, but never anything that would put a patient at risk.

So the 25th of the month is a hard date in your diary from the month before the management period starts, and the fifth business day is the date you check the money arrived.

Note the gap flagged in Part Three: **none of this is on the platform as an engagement yet**, so the invoice run cannot see it.

The other two Medbury strands, which are mostly Lagos.

The aesthetics venture is a jointly held special purpose vehicle agreed in August, with its own name and board, built around **Dr Chinwe Kpaduwa**, who practises in the United States and leaves Nigeria in November. There is a separate Lagos setup mandate and a management fee structure of a monthly base plus a share of collections and margin. Running alongside it is **Medlyfe Introduces**, a themed evening of a fireside chat, a panel and a cocktail reception on **8 October 2026** under the Medlyfe brand, designed as the first edition of a recurring series rather than a launch, so that editions two, three and four inherit the format, the identity, the film library and the guest list on Dr Chinwe's 2027 visits. It carries an eight week campaign behind it. The third strand is the Alameda conversion clinic, which has its own business case and cost base.

Your exposure to these is the diary, the guest list follow up and the money. The evening is in a week.

4.4 Aman HMO

Aman Health is a Takaful health company, which means it is structured on Islamic cooperative insurance principles, historically northern in its footprint and headquartered here, which opened a Lagos office in mid 2026 to drive southern growth. The chief executive is **Abubakar Hassan Muhammad**, who goes by Sadeek Hassan and who sent us the brief, and the contact on our client record is **Zayyad Abdulrahman**, the chief financial officer. They cover around fourteen thousand lives and their brand promise is zero rejected eligible claims.

The engagement is provider network growth in Lagos, taking them from about a hundred and twenty onboarded providers toward three hundred, and the strategy we sold them is not more email. It derives provider targets from where their enrollees actually are, starting with the Lagos branch addresses of the northern corporate accounts they already hold, then builds the network through two channels email cannot reach, which are the Islamic affinity organisations and faith aligned facilities, and direct engagement of senior Lagos specialists who practise across several hospitals so that one relationship extends member access to several facilities at once.

There is a balance outstanding against a due date of 27 September, so it is already past due and it is one of the larger receivables in the firm. The signed commercial was structured as a mobilisation payment plus monthly execution payments, and **the engagement figure on the platform does not obviously reconcile to it**, so before you chase, check one against the other and tell Debo which is right. That reconciliation matters more than the chase, because chasing the wrong number in writing is hard to walk back.

4.5 Solace Healthcare, Games Village

Not yet a client, and the nearest thing to one in the Abuja pipeline.



Dr Oby Clems-Anunwa is a consultant family physician at National Hospital Abuja who is converting four to five built residential units near Games Village into a health facility, phased as day case ambulatory, then inpatient at half capacity, then a centre of excellence, against a monthly revenue target she set herself for year two. We delivered the advisory and the proposal on 14 September.

The finding that drives the whole document is that the converted site on its own does not reach her target inside two years, while the same site plus a small leased diagnostic and clinic spoke, opened near the end of year two, does. So her phase three ambition contains a phase one decision, and that is the argument the proposal is built on. The positioning matters too, because the African Medical Centre of Excellence is under twenty minutes away and does the entire cancer pathway including radiotherapy, priced against Chennai and Istanbul, so Solace sits underneath it at roughly half the flagship tariff serving everyone who cannot reach those prices. We never frame Solace as a competitor to that centre.

The fee is anchored high and built from resourced days plus a programme rate, over a twenty seven month programme. There is a private negotiation guide in the repository, with the floor and the concession ladder in it, and it is never sent to a client under any circumstances.

What you need from this file is the chase. A proposal delivered on 14 September with no decision by 1 October is exactly the kind of thing the Thursday sweep exists to catch.

4.6 The federal and institutional layer

This is the part of Abuja that does not appear on an invoice and decides a great deal.

The Healthcare Federation of Nigeria is the private sector umbrella body, and its President is **Njide Ndili**. Debo is a member rather than in its leadership. In July he offered to write, and then drafted, a **policy brief on the medical internship placement bottleneck** for the federation's leadership to adopt and send to **Professor Iziaq Salako, Minister of State for Health**. It is written in the federation's collective voice so leadership can put it on their own letterhead, and his own foundation role appears only in the signature so as not to overshadow them.

The argument is worth knowing because it is the clearest example of how Debo writes a policy document. Bribery and the pain of centralised posting are symptoms, and the real problem is structural, so the brief separates four root causes and gives each a single matching remedy. The anchor statistic is that roughly two thousand of about six thousand annual medical graduates are stranded against an accredited capacity of around four thousand, which the Medical and Dental Council's Registrar told the Senate, and the council has already said publicly that the fix is state and private hospitals, which means the door is open rather than needing to be forced.

Debo is also President of the **Doctors Foundation for Care**, which is the vehicle through which some of this convening work happens.

What this means for you practically is that institutional correspondence in Abuja is a category of work in its own right, it moves on other people's timelines, and it needs the same chasing discipline as a client. If a brief has gone to a federation president in July and there is no record of whether it reached a minister, that is a commitment with no chase record, and it belongs on the register.

4.7 The regulators and bodies you will be dealing with

You do not need to be an expert on these. You need to recognise the names, know roughly what each one gates, and know that several of them are a short drive from you rather than a letter away.



Body	What it gates	Why it matters to us
Corporate Affairs Commission	Incorporation, filings, RC numbers	Every venture structuring engagement ends here, and the Arabella succession question starts with a certificate
Federal Inland Revenue Service	Company tax, withholding tax remittance	We deduct 5 per cent withholding tax from contractor payments and remit it, which is a real obligation not a formality
Medical and Dental Council of Nigeria	Doctors' registration, internship accreditation	Limited versus full registration decides whether a diaspora clinician can work here, and it is the body at the centre of the housemanship brief
NAFDAC	Medicines, devices, implants	Implant registration came up directly in the Osteon audit
Nigerian Nuclear Regulatory Authority	Radiation sources, imaging, radiotherapy	Needed at design stage rather than after a build, which is why it is a long pole on Solace, and it covers the imaging equipment in a theatre
FCDA and the Area Council development control	Change of use from residential	Both the Solace units and the Lyfe Place compound are residential buildings becoming clinical ones
The FCT health administration	Facility registration in Abuja	The Federal Capital Territory equivalent of the Lagos facility monitoring agency. Confirm the exact route and office with counsel before quoting a timeline to a client
Nigeria Data Protection Commission	Data protection under the Act of 2023	Patient data, waiting lists and any database reactivation. The Act of 2023 superseded the older 2019 regulation, so say NDPA and not NDPR
Controlled drug licensing	Opioids for palliative care	Lodged roughly nine months before a palliative service opens

One correction to carry: the older regulation is often still quoted in documents and conversations. The current law is the Nigeria Data Protection Act 2023.

4.8 The relationships in this city

Debo ran **Cedarcrest Hospitals Abuja** as chief executive, which is the single most useful fact about his standing here. Kola Momodu, our on site lead at Arabella, spent nearly nine years at Cedarcrest. Dr Bola Akinola, our Lagos arthroplasty client, operates at Cedarcrest. The hospital recurs across the file and the goodwill is real.

Dr Oby at Solace is a consultant at **National Hospital Abuja**, which is also where a good deal of federal clinical opinion sits.

Before you draft anything to anybody in this city, ask Debo how he knows them. He usually does, and a cold opening to somebody he worked beside reads badly in a way a late reply never does.

4.9 What good looks like in Abuja by the end of October

The Arabella link has been sent, the surveys have real response counts rather than zeroes, the audit scope is confirmed in writing on both sides, the two documents we need from Dr Chito are in hand, the operations and commercial diagnostic went out by the end of week two, and the weekly brief has gone every week



without being chased.

The visa medical panel question on Lyfe Place has an answer, the sessional fill contradiction has been reconciled in one document, and the schedule of expected government fees for Lagos and Abuja is with Medbury before the commencement date.

The Aman balance has either been paid or has a promised date against it on the commitment register, and Solace has a decision or a written reason for the delay.

Part Five: The rest of the book of work

5.1 The whole book on one page

Client	Where	What we are doing	Status	Fee shape
Arabella Women's Health	Asokoro, Abuja	Diagnostic audit, gate to a management contract	Starts 2 Oct, balance outstanding	Fixed fee, then a share of revenue
Medbury Healthcare Group	Lagos and Abuja	Division setup and management, campus conversion, aesthetics venture	Closed 21 Sep, setup running	Setup in tranches, monthly support, performance share
Haven Paediatric Centre	GRA Ikeja, Lagos	Turnaround, culture build, then the growth mandate	Active to Jan 2027, instalments outstanding	Fixed fee in instalments
Belfiore Medical	Ikoyi, Lagos	Client records system, then the client experience programme	Active, balance outstanding	Fixed fee per phase
Aman HMO	Abuja and Lagos	Lagos provider network growth	Active, balance past due	Mobilisation then monthly execution
Osteon Clinics	Amuwo Odofin, Lagos	Organisational audit	Fieldwork done, report due	Carried by CFA, not billed
Havana Specialist Hospital	Lagos	Board governance framework	Complete and settled in full	Fixed fee, board partner concession
House of Refuge	Lekki, Lagos	Management retainer and a fundraising mandate	Running, not on the platform	Monthly retainer plus a success fee
Dr Chinwe Kpaduwa	United States and Lagos	Her visit, and the aesthetics venture around her	Planning, she leaves in November	Inside the Medbury structure
Covally, Connexum, Cureva, Cooked Indoors, Osibil, Doctoora	Lagos	Founder and venture portfolio	Mixed	Mostly internal or planning



Client	Where	What we are doing	Status	Fee shape
Priscilla Specialist Hospital	Ajah, Lagos	Management and digital transformation	Cancelled, client still active	Was a large transformation mandate

5.2 Osteon Clinics

Dr Bola Akinola is a United Kingdom trained arthroplasty surgeon in Amuwo Odofin, and this started in July as a positioning engagement before becoming an organisational audit, because the strategy we wrote was a set of promises about the business rather than about the surgeon and none of them could be executed until the organisation underneath was known.

The thesis we put to him in writing, rather than saving for the report, is that the practice is bigger than the company. A meaningful share of his income is earned in other people's theatres and invoiced outside Osteon, while Osteon carries a fixed cost base for a surgeon who is frequently elsewhere.

The method is worth knowing because we reuse it. Ten real cases, picked on the morning of the visit, each followed through a twenty two link chain from first enquiry to current outcome, so that every gap found is specific rather than theoretical.

Fieldwork was on 17 September and the report is due early October. **There are still zero survey responses and zero uploads**, two weeks on, and there is no evidence the cover email was ever sent. His email and phone are not in our records. That is a chase, not a mystery.

The audit is carried at our cost and the documents say nothing about money at all, deliberately, because writing that it is free would price every later workstream against zero. If he raises it, the position is that the audit is the groundwork for the work that follows rather than a favour.

5.3 Belfiore Medical Aesthetics

Dr Uju Rapu is the owner and chief executive of an aesthetics clinic on Shoreline Drive, Ikoyi. Two things run. The client records system with the data protection groundwork behind it is phase one, and **a balance on it fell due on 30 September**. The client experience programme for three front desk staff ran on 25 and 26 September, priced per head, and **that second invoice does not exist on the platform at all**, which means it is not being chased by anything.

Two things to carry about this client. She has asked for a refund before, on an invoice that was silent on the point, so every Belfiore invoice carries the terms block stating that fees are non refundable, that a substitution is free and that one reschedule is included. And Belfiore has its own document design rather than our sober audit template, because she is a luxury client and the house template reads wrong for her.

5.4 Haven Paediatric Centre

A board led paediatric and neonatal turnaround in GRA Ikeja, now in its growth phase. The founders set the mandate themselves and pushed the timeline, which is roughly a tripling of monthly revenue against a stated margin floor within six months, led by the neonatal unit into the paediatric high season from November to February. The targets are in the growth strategy document and the deck.

Two findings from that work are worth remembering because they come up in conversation. Most of their two thousand four hundred line tariff was priced at zero, and the lines that were priced sat well below the market, so a large part of the plan is a repricing in two tranches across the year. And we told them not to host obstetricians' deliveries, because obstetricians keep their own delivery revenue and will not refer into a unit that competes with them, so instead we built an antenatal pathway that pre books neonatal admissions and invited them to tour the unit.



The engagement fee is scheduled as three equal instalments due on 25 August, 23 September and **22 October**, and the first two are the overdue part. Kabir Aregbesola is the contact and Usman is copied.

5.5 Havana Specialist Hospital

A Lagos hospital owned by two founding families, both founders now late, with a board of eight split four to four. Debo sits on it as an **independent non executive director** while CFA was also a paid adviser setting up the board operating model, and that dual position is handled by disclosure and recusal rather than by pretending it does not exist. The fee was approved by the remuneration committee with him recused.

The engagement is complete and every invoice on it is paid, which makes it the only file in the book with nothing outstanding. The board politics are genuinely sensitive, because one family convened the board and nominated the non executives, so the other family perceives the board as tilted. Do not discuss this file outside the firm, at all, and do not minute anything from it loosely.

The operating partner **Afya Care** runs the hospital day to day through a joint governance committee, and Favour Olewunyi is the contact on that side.

5.6 House of Refuge, Freedom Foundation

Pastor Tony Rapu's faith integrated rehabilitation facility in Lekki. The readiness project is complete, and we now run management support on a **monthly retainer** plus a **fundraising mandate charged as a share of funds raised**, which is a pure contingent fee.

Note the direction of travel here, because it is deliberate. Alongside the Clearview work, Debo is moving the firm's commercial model toward performance and success fees rather than resourced days. Neither of these two strands is on the platform, so the monthly retainer is not visible to the invoice run.

5.7 Kemi Balogun's group, and ile

A family group across education, energy, agriculture, healthcare and fashion, for which we delivered a holding company structuring note with five options, a cap table and founders' agreement for the healthcare venture, an instruction brief for her appointed counsel, a one page terms sheet for her clinical co founder, and a rollout plan.

The healthcare venture is **ile**, and in September it was inverted. It is a **residential eldercare business**, and home care plus virtual consultations are the bridge that funds the build, proves the care standard and produces the admissions pipeline that fills the beds on opening day. The structural call that came out of that is to separate the building from the business, because a single company cap table would mean our equity stake was a stake in real estate she funded, which is neither what we earned nor what she intended to give.

Status is that the clinical co founder has agreed terms in principle, counsel is instructed and drafting, there is no entity yet, and the waiting list has been live since mid September. For a residential business the waiting list is the occupancy plan rather than a mailing list.

5.8 The prospects worth knowing by name

Duchess International Hospital, a hundred bed hospital in Ikeja GRA. The proposal, an HR prospectus and a leadership assessment invitation have all gone to Mrs Ladipo, with the fee payable in three parts.

Clearview Fertility and Hospital, Dr Kunle Ajayi in Lekki. An inbound fertility medical tourism thesis, where the argument is that the hospital is what turns a clinic into a destination.

Iwosan Lagoon and the IASO Medipark, Fola Laoye, whom Debo knows from the Doctoora raise that led to his isolation centre role. A warm approach timed around the opening of a hundred and forty bed facility.

Afya Care, Tosin Runsewe, who knew Debo at Evercare and Cedarcrest and is now a board chairman, with the target being a fractional group level function.



Dennis Ashley, Dr Chima Oti, endoscopy suites in Victoria Island, under a management arrangement priced as a share of collections, with the diagnostic audit carried at our cost. The audit site is live but there have been zero uploads and zero survey responses, and the promised non disclosure agreement does not exist yet. Worth noting that this one has been deferred, so check with Debo before chasing.

Priscilla Specialist Hospital in Ajah, where the engagement was cancelled and the relationship is not.

5.9 The products, and what each needs from the office

Maarova. Leadership assessment and coaching. Active cohorts run for client leadership teams, and the Founding Circle is a hand picked group of clinical leaders given complimentary places. Coaching records and Founding Circle applications are outside your access because they are personal development records.

CadreHealth. The workforce platform, holding over ten thousand professional records, of which about a thousand are registered with a password. Two cautions that matter if you ever touch a send. The platform's historic imports put the title in the first name field and the middle name in the last name field, so roughly a quarter of greetings built as doctor plus last name are wrong, and there is a helper in the codebase that exists specifically to get a salutation right. Separately, a bulk send went out historically with an eleven per cent hard bounce rate because the mail provider's webhook was never configured, so nothing was ever suppressed.

Mezo Health. A private practice network sold to doctors aspirationally as their own practice without building one. Nine hundred and eighty eight eligible doctors were seeded with accounts on 22 September, only fourteen have been claimed, and **every claim link expires around 22 October**. If a campaign is going to run after that date, the links have to be reissued first. That date should be in your diary.

5.10 The family estate

Debo runs facility management for the family estate, which is twelve units across three buildings, two on Banana Island and one on Osborne Foreshore. It sits on the founder side of the platform with link based surfaces for tenants and for the ground team.

The thing to understand is that a generator run is the billable event and rates are frozen onto each run when it is costed, so moving the rate card never restates a past charge, and a unit keeps a ledger rather than a balance, so corrections are adjustment rows rather than edits. One legal obligation is outstanding: the Lagos Tenancy Law requires a separate service charge receipt and a written account of how the money was spent at least every six months, and that is not built yet.

Part Six: Money, and the chase

6.1 Where the numbers are, and why they are not in here

Open **/finance/invoices** on the platform. Your role reaches it, it is read only, and it is current, which no document can be. Filter to anything past its due date and that is your chase list.

What you will find when you first look at it, as at the beginning of October, is **four client balances outstanding and three of them already past due**. Two fell due on 30 September and one in late September. Only the newest one is not yet due. That shape is the problem rather than any single number, because three simultaneous overdue balances means the chase was not happening rather than that one client is slow.

Then there is money that is contracted and **not invoiced at all**, which is worse, because nothing on the platform is tracking it. The Medbury setup tranches and the monthly support fee from month six, both House of Refuge strands, and the Belfiore client experience programme that was delivered in September. Getting those engagements onto the platform is what makes the invoice run able to see them.



Fees, rates and client commercials are not written into this pack by Debo's instruction. When you need a figure, read it off the platform on the day you need it, and when you put one in a message to a client, copy it from the invoice rather than from memory or from an older document.

6.2 How the chase works

The invoice run is the 1st and the overdue chase is the 15th, and both are dates rather than decisions, because chasing money is uncomfortable and anything uncomfortable that depends on somebody choosing to do it does not happen.

A chase is only finished when it is recorded. The register wants the date the client actually promised to pay, not the fact that you called, because a promise with a date can be chased against and a note saying you called cannot. Anything disputed gets escalated rather than chased again.

Your read only access to finance exists for exactly this. You will see invoice status, amounts and payment dates. You will not see consultant rates or agent commissions and you cannot change a figure, which is deliberate.

6.3 What goes on every invoice

Fees are **non refundable**. A **substitution is free**, so if the named person cannot attend a training or a session, somebody else may take the place at no cost. **One reschedule** is included.

That block exists because a client asked for a refund on an invoice that was silent on the point, and silence is what made the conversation possible. It goes on every invoice, including ones raised in a hurry.

The billing entity is **Consult for Africa Management Services Limited**. Value added tax is 7.5 per cent and is shown separately. Where we pay an independent contractor we deduct 5 per cent withholding tax at source and remit it, and the contractor is entitled to the credit note, so that is a real obligation rather than a formality.

Bank details belong on the invoice template rather than in a message. There is both a Zenith account used on the Havana invoices and a separate First Bank transfer instruction in the document folder, so confirm with Debo which account is current before you send a payment instruction to anybody.

6.4 The payment discipline we now ask for

The Medbury memorandum is the template for how Debo wants payment structured from here, because late payment puts delivery at risk rather than merely annoying us. The terms that actually work, in the order they work, are a **standing payment instruction** so no human approves it monthly, **one month held as a deposit** and applied to the final month, and **pay the undisputed part on the due date** while the disputed part is notified within five business days with reasons. Interest, suspension rights and breach clauses matter less because they are used last.

When you are preparing a new engagement letter or a retainer, those three are the ones to make sure survive the client's redline.

Part Seven: How we write, and what never leaves the building

7.1 The register

We write to people rather than about them. Our clients are senior, most are clinicians, and several know Debo personally, which sets the register for everything.

In a client document the client is **you** and the firm is **we**. First person singular is reserved for Debo's own letters and for the judgements he is personally making. Name the client's people by role and warmly, so a finding about a system never reads as a finding about a person.



We do not perform enthusiasm, we do not use exclamation marks, and we do not write a sentence that a consultant reading it on a phone between cases would find embarrassing.

Say what we cannot tell them. Candour about the limits of evidence reads as integrity, and it protects the firm.

7.2 The things that make writing read as generated

Debo rejected a draft in his own name in September with the note that it read as machine written, and the content had been right. The problem was the furniture. Avoid all six of these.

- Three paragraphs opening with matched constructions
- The short pivot sentence, where a long line is followed by a four word correction
- An epigram at the end of every paragraph, because one aphorism is a voice and four is a machine
- A line commenting on the document itself
- Paragraphs all the same length, and justified text
- Enumerating in prose with first, second and third

What works instead is opening with what was actually done rather than a framing sentence, varying paragraph length hard, letting a blunt opinion sit without a line explaining it, and cutting about a third of the words.

His own rhythm is long connected sentences joined with because, so and which, explaining as they go. Staccato declaratives and constructions of the form not this but that are the tell, even when every word is right.

7.3 Two hard rules on characters and names

Never use an em dash. Not in a document, not in a headline, not in a message, not in interface text. Use a comma, a full stop, a colon, or restructure the sentence.

Never name our platforms in client body copy. Describe what is done. The only exception is an appendix the client has specifically asked for.

7.4 Confidentiality and the anonymisation standard

Everything in this pack is internal. When our work is described publicly, a client is identified by sector, setting, scale and stage, never by name, owner, district, or any combination of details that would identify them in a market this small. Ratios may be published and absolute naira figures tied to an identifiable client may not.

Two files are excluded from external use absolutely, which are Haven's clinical incident material and Havana's family politics.

7.5 Before you write to anybody outside the firm

Ask Debo how he knows them. He usually does, and the cold register insults an old relationship.

For outreach to senior clinicians, consultants, emeritus physicians and the diaspora, there is a separate set of rules about what to say and what never to say, and it is worth reading before your first campaign rather than after it.

7.6 Email, and the two mistakes that have been made before

The sender of record is **hello@consultforafrica.com**, and that matters because it is the mailbox Debo checks, so anything sent as something else leaves no evidence where he will look for it.



Bulk and outreach sending goes through the transactional mail provider's interface only. The ordinary office mail system throttles batches and starts refusing them partway through, which has happened, and a half sent campaign is worse than an unsent one.

And before any bulk send, send one real message to a real destination and check that the record actually arrived, because a live page can sit in front of a dead endpoint and a success code can sit in front of a parser that read nothing.

7.7 Deliverables

Client documents are built as PDFs from scripts in the repository and land in the documents folder, which is how a figure in a summary cannot disagree with the figure in the full document. They are not web pages and they are not pasted into email bodies.

For a proposal or a pitch to a principal, the deliverable is a landscape deck and the prose document is the leave behind. Reports, letters and memoranda stay as prose.

Part Eight: Your first ninety days

Week one, Friday 2 October to Thursday 8 October

You start on **Friday 2 October**, which is also Arabella day one in Asokoro, so your first day and the firm's largest starting engagement begin together. Kola is the one who has to be on site, not you, but read Part Four before Friday rather than after it. The other fixed date in the week is **Thursday 8 October**, which is the Medlyfe evening in Lagos.

The list below is already on your desk on the platform as seven tasks, dated across these five working days, each with a brief and a definition of done.

- Log in, change your password, read your desk
- Get Dr Chito's email and phone, put them on the client record, send the ArabellaProject link to her and to Tolu
- Clear the review backlog with Debo: two submitted tasks and three ungenerated Monday packs
- Rebuild the commitment register from the meeting records and message threads since July
- Open the decisions register with the open items in this pack, starting with the visa medical panel question and the sessional fill contradiction
- Confirm Kola is on site Friday and for the four weeks after
- Chase every past due balance on the Invoices surface, and record a promised date for each
- Sit with Debo on Arabella and the Medbury division, as his letter asks

Weeks two to four

- Get the Medbury division and both House of Refuge strands onto the platform as engagements
- Reconcile the Aman engagement figure against the signed commercial
- Get the office inventory count onto the register, writing the worked example first so the standard is visible before the work starts
- Fill the missing client contacts: Osteon, Havana, and anywhere else a send would fail
- Take over the Monday pack as a reviewed product, not a chased one
- Hold the first monthly evaluation with Abigail on the 28th, conversation before the form
- Mezo claim links expire around 22 October. Decide with Debo whether they are reissued
- Haven instalment three falls due 22 October



Month two

Own the rhythm rather than operating it. By the end of month two the Monday pack should be going out on Friday evenings without anyone asking, the Thursday sweep should be finding fewer dropped items because fewer are being dropped, and the commitment register should be the thing the partner meeting runs off.

Start preparing meeting packs ahead of the meeting rather than minutes afterwards. A pack that tells Debo what he is walking into is worth more than a record of what he said.

Month three

The withheld access list gets revisited. What earns it is an empty review backlog, a current chase register, and work coming back finished because what went out was good.

Appendix A: The directory

Confirm titles and reporting lines with Debo in your first fortnight, because some of these are practice or faculty roles rather than full time posts.

Person	Role	Contact
Dr Debo Odulana	Founding Partner	debo.odulana@consultforafrica.com
The firm	Sender of record for anything outbound	hello@consultforafrica.com
Abigail Oladejo	Administrative Assistant, reports to you	abigail.ayomide04@gmail.com
Toluwanimi Olayeni, Nimi	Marketing and communications lead	On the platform
Ejiro Alechenu	Marketing	ejiroalechenu0@gmail.com
Dr Olufunbi Okeleye	Engagement Manager	engagement.management@consultforafrica.com
Oludayo Olugbamigbe	HR Practice Lead	Confirm with Debo
Kola Momodu	Arabella on site Project Owner	Confirm with Debo
Dr Chito Nwana	Principal, Arabella Women's Health	Missing. Get this first
Tolu	Chief of Staff, Arabella	Confirm on Friday
Dr Itunu Akinware	Group Chief Executive, Medbury	On the client record
Dr Chinwe Kpaduwa	Aesthetics clinical partner, Medbury venture	On the client record
Zayyad Abdulrahman	Chief Financial Officer, Aman HMO	On the client record
Dr Uju Rapu	Chief Executive, Belfiore Medical	On the client record
Dr Bola Akinola	Osteon Clinics	Missing
Kabir Aregbesola	Haven Paediatric Centre, main contact	kabir@aurorahills.co
Usman Gumi	Haven, copied on invoices	usman.g@aurorahills.co



Person	Role	Contact
Abisodun Alli, Shakirah Saliu, Dr Odedina, Ogochukwu Odum	Haven founders	abisodunalli@yahoo.com, shakirahsaliu@gmail.com, gbajoodedina@gmail.com, odumogo@gmail.com
Mr Ugo Nwokoro	Executive Director, Havana	Needs a contact record
Mrs Nneka Chukwubuisi Solomon	Head of Finance, Havana, processes payment	Needs a contact record
Favour Olewuenyi	Afya Care, runs the joint governance committee report	favour.olewuenyi@afya.care
Dr Oby Clems-Anunwa	Solace Healthcare, Games Village	Confirm with Debo
Njide Ndili	President, Healthcare Federation of Nigeria	Through Debo only

A note on one address

Debo holds two platform accounts and only the consultforafrica.com one reaches the elevated pages. When he says he cannot see something, check which account he is signed into before you check anything else.

Appendix B: Glossary

Term	What it means here
Audit, organisational	Our structured diagnostic across ten domains of a facility, ending in a report and a set of board decisions
Build and operate	We design and set up a facility or venture, then run it under a management agreement
CAC	Corporate Affairs Commission. Incorporation and company filings
CSSD	Central sterile services department. Where instruments are sterilised, and a catastrophic risk point in surgery
Day case	A procedure where the patient goes home the same day
DoA	Delegation of authority. The written list of what a board keeps and what it delegates
EBITDA	Earnings before interest, tax, depreciation and amortisation. The operating profit measure in every one of our models
EMR	Electronic medical record
FCDA and AMMC	Federal Capital Development Authority and the area municipal council. Development control, including change of use
Fractional leadership	A senior operator placed part time into a client, such as an interim chief operating officer



Term	What it means here
HEFAMAA	The Lagos State facility monitoring and accreditation agency. Abuja has its own route
HMO	Health maintenance organisation. A health insurer, and the payer behind much of a hospital's revenue
MDCN	Medical and Dental Council of Nigeria. Registers doctors and accredits internship places
Medipark	A campus of several independent clinical businesses sharing a building and its services
MOU	Memorandum of understanding
NAFDAC	The regulator for medicines, devices and implants
NDPA 2023	Nigeria Data Protection Act, which replaced the 2019 regulation. Say NDPA, not NDPR
NICU	Neonatal intensive care unit. Three cots at Haven, and the highest value admission in the building
NNRA	Nigerian Nuclear Regulatory Authority. Radiation sources, imaging and radiotherapy
Panel, insurance	The list of facilities an insurer will pay. Being on it is the commercial gate
RC number	A company's registration number at the Corporate Affairs Commission
Resourced days	How we price delivery work, as named people for a number of days at published rates, plus a programme rate
Secondment	Someone employed by one party and directed by another. The structure of the two Medbury operating officers
Sessional	A doctor who books a session in a clinic rather than being employed by it
SPV	Special purpose vehicle. A company formed for one venture
Takaful	Islamic cooperative insurance. The structure Aman is built on
Tariff	A facility's price list, line by line
VMI	Vendor managed inventory. The supplier holds the stock and is paid on use, which frees working capital
WHT	Withholding tax. We deduct 5 per cent from contractor payments and remit it

Appendix C: The standing checklists

The Monday partner meeting pack, prepared by Friday evening

- Every active engagement, one line each, with status
- Anything that slipped since last week, and why
- Decisions needed from the partners, with options and a recommendation
- The diary for the week ahead
- Receivables, with what was promised and by whom
- Sent to attendees Friday evening, not Monday morning

The Thursday pipeline sweep



- Every lead with no contact in fourteen days chased, or a written reason on it
- Every proposal sent more than a week ago followed up
- Anything needing a partner rather than you on the commitment register with a name against it
- Draft proposals that were never sent either sent or closed

The invoice run, 1st of the month

- Every engagement with billable work has an invoice raised and approved
- Terms block present: non refundable, free substitution, one reschedule
- Tax shown separately, correct entity name, correct bank details
- Anything that cannot be invoiced has a written reason
- The list of what went out is with Debo

The overdue chase, 15th of the month

- Every invoice past due chased this month
- A promised payment date recorded against each one
- Disputes escalated rather than chased again

Minutes of any meeting

- Decisions, each with a name and a date
- Commitments, split by whether we owe it or they do, onto the register
- Anything that became a task, assigned with a brief and a definition of done
- Out the same day, because a minute that arrives on Thursday is a record and one that arrives within the hour is a tool

Appendix D: Where things live

Client documents are PDFs in the repository's documents folder, organised by client. Each one is generated by a script, so a figure cannot drift between a summary and a full document. The ones most worth reading in your first fortnight are the Arabella information request, the Medbury division memorandum and its one page summary, the Haven growth strategy, and the marketing handbook, which covers the four brands and the rules in more depth than Part One does here.

Client facing project links are single unindexed pages that carry everything a client needs instead of six attachments. Two are live, which are the Arabella one and the Osteon one.

Surveys and their responses sit on the platform under the admin surveys reader rather than in separate places, and response counts are the thing to check weekly during an audit.

Office of the Founding Partner, Consult for Africa. Internal. Prepared 1 October 2026, revised 2 October 2026 to narrow the Arabella scope to the diagnostic audit. If anything in this pack turns out to be wrong, say so, and it gets corrected rather than worked around.